

COMMITTEE ON REGULATORY AFFAIRS AND GOVERNMENT EFFICIENCY
SENATE AMENDMENTS TO H.B. 2251
(Reference to House engrossed bill)

Amendment instruction key:

[GREEN UNDERLINING IN BRACKETS] indicates text added to statute or previously enacted session law.

[Green underlining in brackets] indicates text added to new session law or text restoring existing law.

[GREEN STRIKEOUT IN BRACKETS] indicates new text removed from statute or previously enacted session law.

[Green strikeout in brackets] indicates text removed from existing statute, previously enacted session law or new session law.

<<Green carets>> indicate a section added to the bill.

<<Green strikeout in carets>> indicates a section removed from the bill.

1 The bill as proposed to be amended is reprinted as follows:

2 Section 1. Section 36-751, Arizona Revised Statutes, is amended to
3 read:

4 36-751. Definitions

5 In this article, unless the context otherwise requires:

6 1. "Department" means the department of health services.

7 2. "Director" means the director of the department ~~of health~~
8 ~~services~~.

9 3. "LICENSED MIDWIFE" MEANS A MIDWIFE WHO IS LICENSED PURSUANT TO
10 THIS ARTICLE.

11 ~~3.~~ 4. "Midwife" means a person who delivers a baby or provides
12 health care related to pregnancy, labor, delivery and postpartum care of
13 the mother and her infant.

14 Sec. 2. Section 36-755, Arizona Revised Statutes, is amended to
15 read:

16 36-755. Powers and duties of the director

17 A. The director may adopt rules necessary for the proper
18 administration and enforcement of this article.

19 B. The director shall, by rule:

20 1. Define and describe, consistent with this article and the laws
21 of this state, the duties and limits of the practice of midwifery.

22 2. Adopt standards with respect to the practice of midwifery
23 designed to safeguard the health and safety of the mother and child.

24 3. Establish the criteria for granting, denying, suspending and
25 revoking a license in order to protect the health and safety of the mother
26 and child.

27 4. Describe and define reasonable and necessary minimum
28 qualifications for LICENSED midwives, including:

29 (a) The ability to read and write.

1 (b) Knowledge of the fundamentals of hygiene.

2 (c) The ability to recognize abnormal or potentially abnormal
3 conditions during pregnancy, labor and delivery and following birth.

4 (d) Knowledge of the laws of this state concerning reporting of
5 births, prenatal blood tests and newborn screening and of the rules
6 pertaining to midwifery.

7 (e) Education requirements.

8 (f) Age requirements.

9 ~~[C. THE DIRECTOR SHALL CONSIDER THE MOST RECENT MIDWIFERY JOB~~
10 ~~ANALYSIS ISSUED BY THE NORTH AMERICAN REGISTRY OF MIDWIVES WHEN ADOPTING~~
11 ~~OR AMENDING RULES RELATING TO THE MIDWIFERY SCOPE OF PRACTICE.]~~

12 Sec. 3. Title 36, chapter 6, article 7, Arizona Revised Statutes,
13 is amended by adding sections 36-761, 36-762 and 36-763, to read:

14 36-761. Prescription drugs; dispensing and administration
15 authority; storage; documentation

16 A. A LICENSED MIDWIFE WHO, IN ADDITION TO THE REQUIREMENTS FOR
17 INITIAL LICENSURE, HAS PROOF OF COMPLETION OF A PHARMACOLOGY COURSE OF AT
18 LEAST EIGHT CONTINUING EDUCATION UNITS FROM A MIDWIFERY EDUCATION
19 ACCREDITATION COUNCIL-ACCREDITED INSTITUTION OR A REGIONALLY ACCREDITED
20 INSTITUTION MAY DISPENSE AND ADMINISTER THE FOLLOWING MEDICATIONS,
21 THERAPIES AND DEVICES WITH THE FOLLOWING LIMITATIONS:

22 1. ANTIBIOTICS RELATED TO GROUP B STREPTOCOCCUS PROPHYLAXIS
23 CONSISTENT WITH THE UNITED STATES CENTERS FOR DISEASE CONTROL AND
24 PREVENTION GUIDELINES OR OTHER CURRENT EVIDENCE-BASED GUIDELINES.

25 2. THE FOLLOWING ANTIHEMORRHAGIC MEDICATIONS FOR PREVENTING AND
26 TREATING POSTPARTUM HEMORRHAGE AS DEFINED BY THE AMERICAN COLLEGE OF
27 OBSTETRICIANS AND GYNECOLOGISTS:

28 (a) OXYTOCIN FOR PREVENTING UTERINE ATONY DURING OR IMMEDIATELY
29 FOLLOWING THE THIRD STAGE OF LABOR ACCORDING TO CURRENT EVIDENCE-BASED
30 STANDARDS OF CARE.

31 (b) METHYLERGONOVINE.

32 (c) CARBOPROST TROMETHAMINE.

33 (d) MISOPROSTOL.

34 (e) TRANEXAMIC ACID.

35 A LICENSED MIDWIFE MAY NOT USE THE ANTIHEMORRHAGIC MEDICATIONS SPECIFIED
36 IN THIS PARAGRAPH TO INDUCE OR AUGMENT LABOR. THE LICENSED MIDWIFE SHALL
37 INITIATE A TRANSFER OF CARE IF AN ANTIHEMORRHAGIC MEDICATION IS
38 ADMINISTERED FOR MANAGING HEMORRHAGE.

39 3. PROPHYLACTIC OPHTHALMIC MEDICATIONS FOR NEWBORN EYE CARE.

40 4. INTRAVENOUS FLUIDS FOR THE FOLLOWING:

41 (a) MEDICATION ADMINISTRATION.

42 (b) ROUTINE FLUID ADMINISTRATION.

43 (c) TREATING HYPOVOLEMIC SHOCK WHILE AWAITING EMERGENCY MEDICAL
44 SERVICE.

1 5. VITAMIN K PROPHYLAXIS FOR NEWBORNS.

2 6. RHO IMMUNE GLOBULIN.

3 7. TOPICAL, INTRAMUSCULAR OR SUBCUTANEOUS LOCAL ANESTHETICS FOR
4 POSTPARTUM REPAIR OF FIRST AND SECOND DEGREE TEARS, LACERATIONS OR
5 EPISIOTOMIES. THIS PARAGRAPH DOES NOT INCLUDE ANY SCHEDULE I CONTROLLED
6 SUBSTANCE.

7 8. OXYGEN AND COMPRESSED AIR FOR FETAL OR MATERNAL DISTRESS AND
8 INFANT RESUSCITATION.

9 9. EPINEPHRINE FOR NEONATAL RESUSCITATION CONSISTENT WITH THE
10 NEONATAL RESUSCITATION PROGRAM GUIDELINES AND TO TREAT MATERNAL ALLERGIC
11 REACTIONS.

12 10. GLUCOSE GEL ADMINISTERED ORALLY FOR NEONATAL HYPOGLYCEMIA.

13 11. VITAMINS AND MINERALS FOR CORRECTING ELECTROLYTE IMBALANCES,
14 INCLUDING B VITAMINS, CALCIUM, MAGNESIUM, DIETARY SUPPLEMENTS, HOMEOPATHIC
15 REMEDIES, PLANT SUBSTANCES THAT ARE NOT DESIGNATED AS PRESCRIPTION DRUGS
16 OR CONTROLLED SUBSTANCES, AND OVER-THE-COUNTER MEDICATIONS.

17 12. RESUSCITATION SUPPLIES AND EQUIPMENT ACCORDING TO THE CURRENT
18 NEONATAL RESUSCITATION PROGRAM ALGORITHM.

19 13. MEDICAL SUPPLIES AND EQUIPMENT NEEDED TO ADMINISTER THE
20 MEDICATIONS PRESCRIBED IN THIS SUBSECTION.

21 14. ELECTRONIC BREAST PUMPS, GLUCOSE MONITORS, COMPRESSION
22 STOCKINGS AND PREGNANCY SUPPORT BELTS OR DEVICES.

23 B. ~~[A LICENSED MIDWIFE MAY ADMINISTER OTHER PRESCRIPTION~~
24 ~~MEDICATIONS AS PRESCRIBED BY A PHYSICIAN WHO IS LICENSED PURSUANT TO TITLE~~
25 ~~32, CHAPTER 13, 14 OR 17, A REGISTERED NURSE PRACTITIONER WHO IS LICENSED~~
26 ~~PURSUANT TO TITLE 32, CHAPTER 15 OR A PHYSICIAN ASSISTANT WHO IS LICENSED~~
27 ~~PURSUANT TO TITLE 32, CHAPTER 25.]~~ A PHARMACIST WHO DISPENSES PRESCRIPTION
28 MEDICATIONS PRESCRIBED PURSUANT TO THIS SUBSECTION TO A LICENSED MIDWIFE
29 IS NOT LIABLE FOR ANY ADVERSE REACTION THAT IS CAUSED BY THE LICENSED
30 MIDWIFE'S METHOD OF USE.

31 C. A LICENSED MIDWIFE MAY LAWFULLY OBTAIN, TRANSPORT, ADMINISTER
32 AND POSSESS ADEQUATE QUANTITIES OF THE MEDICATIONS PRESCRIBED IN
33 SUBSECTION A OF THIS SECTION AND THE EQUIPMENT NORMALLY REQUIRED TO
34 ADMINISTER THOSE MEDICATIONS. THE LICENSED MIDWIFE SHALL:

35 1. STORE THE MEDICATIONS PRESCRIBED IN SUBSECTION A OF THIS SECTION
36 AS DIRECTED BY THE MANUFACTURER. A LICENSED MIDWIFE MAY NOT ADMINISTER A
37 MEDICATION TO ANY PERSON AFTER THE LISTED EXPIRATION DATE.

38 2. RECORD IN THE PATIENT'S CHART EACH USE OF MEDICATION AND THE
39 MEDICATION'S LOT NUMBER AND EXPIRATION DATE.

40 36-762. Arizona midwifery advisory committee; membership;
41 duties; sentinel events; definition

42 A. THE ARIZONA MIDWIFERY ADVISORY COMMITTEE IS ESTABLISHED IN THE
43 DEPARTMENT TO ASSIST IN EXAMINING APPLICANTS FOR A MIDWIFERY LICENSE, IF
44 REQUESTED, TO COLLABORATE WITH AND ASSIST THE DIRECTOR IN DISCIPLINARY

1 MATTERS, IF REQUESTED, AND TO PERFORM ANY OTHER DUTIES PRESCRIBED IN THIS
2 ARTICLE.

3 B. THE DIRECTOR SHALL APPOINT THE MEMBERS OF THE ADVISORY
4 COMMITTEE, WHICH SHALL BE COMPOSED OF NOT MORE THAN ELEVEN OF THE
5 FOLLOWING MEMBERS:

6 1. AT LEAST FIVE LICENSED MIDWIVES.

7 2. TWO CERTIFIED NURSE MIDWIVES WHO ARE LICENSED PURSUANT TO TITLE
8 32, CHAPTER 15.

9 3. TWO PHYSICIANS WHO ARE LICENSED PURSUANT TO TITLE 32, CHAPTER
10 13, 14 OR 17[.] ~~[AND WHO ARE BOARD CERTIFIED IN OBSTETRICS, PEDIATRICS OR~~
11 ~~FAMILY MEDICINE] [ONE OF WHOM IS BOARD-CERTIFIED IN OBSTETRICS AND ONE OF~~
12 ~~WHOM IS BOARD-CERTIFIED IN PEDIATRICS].~~ THE DIRECTOR SHALL SOLICIT
13 PHYSICIAN APPLICATIONS FROM STATE MEDICAL SOCIETIES AND ASSOCIATIONS.

14 4. ONE CONSUMER OF MIDWIFERY CARE.

15 C. ADVISORY COMMITTEE MEMBERS ARE NOT ELIGIBLE FOR COMPENSATION OR
16 REIMBURSEMENT OF EXPENSES.

17 D. THE ADVISORY COMMITTEE SHALL MEET AT LEAST FOUR TIMES EACH YEAR,
18 ONCE PER QUARTER.

19 ~~[E. THE ADVISORY COMMITTEE SHALL MAKE RECOMMENDATIONS TO THE~~
20 ~~DIRECTOR WHEN ADOPTING OR AMENDING RULES RELATING TO THE MIDWIFERY SCOPE~~
21 ~~OF PRACTICE.]~~

22 ~~F. A LICENSED MIDWIFE WHO IS INTERESTED IN MODIFYING THE MIDWIFERY~~
23 ~~SCOPE OF PRACTICE MUST SUBMIT A REPORT TO THE ADVISORY COMMITTEE THAT~~
24 ~~CONTAINS AT LEAST THE FOLLOWING:~~

25 ~~1. A DESCRIPTION OF THE ISSUE AND WHY AN ADDITION TO THE MIDWIFERY~~
26 ~~SCOPE OF PRACTICE IS NECESSARY, INCLUDING THE EXTENT TO WHICH CONSUMERS~~
27 ~~NEED AND WILL BENEFIT FROM A LICENSED MIDWIFE'S ABILITY TO PROVIDE THIS~~
28 ~~ADDITIONAL HEALTH CARE SERVICE.]~~

29 ~~2. THE AVAILABLE EVIDENCE-BASED RESEARCH THAT DEMONSTRATES THAT~~
30 ~~LICENSED MIDWIVES ARE COMPETENT TO PERFORM THE PROPOSED HEALTH CARE~~
31 ~~SERVICE.]~~

32 ~~3. THE EXTENT TO WHICH A CHANGE IN THE MIDWIFERY SCOPE OF PRACTICE~~
33 ~~MAY HARM THE PUBLIC, INCLUDING THE EXTENT TO WHICH A CHANGE IN THE SCOPE~~
34 ~~OF PRACTICE WILL RESTRICT ENTRY INTO THE PRACTICE OF MIDWIFERY.]~~

35 ~~G. IF A MAJORITY OF A QUORUM OF THE ADVISORY COMMITTEE MEMBERS~~
36 ~~VOTES TO APPROVE A CHANGE IN THE MIDWIFERY SCOPE OF PRACTICE PURSUANT TO~~
37 ~~SUBSECTION F OF THIS SECTION, THE ADVISORY COMMITTEE SHALL PROVIDE THE~~
38 ~~DIRECTOR WITH ITS RECOMMENDATIONS FOR A POSSIBLE RULE CHANGE.]~~

39 ~~[H.] [E.]~~ THE ADVISORY COMMITTEE SHALL CONDUCT A REVIEW OF
40 SENTINEL EVENTS OCCURRING DURING THE COURSE OF PRENATAL, INTRAPARTUM OR
41 POSTPARTUM CARE PROVIDED BY LICENSED MIDWIVES IN THIS STATE.

1 ~~[F.]~~ [E.] THE ADVISORY COMMITTEE MAY CONVENE EMERGENCY MEETINGS
2 WHEN A CASE OR SENTINEL EVENT REQUIRES TIMELY REVIEW OR ACTION TO PROTECT
3 THE PUBLIC HEALTH AND SAFETY.

4 ~~[J.]~~ [G.] FOLLOWING THE REVIEW OF A SENTINEL EVENT, THE ADVISORY
5 COMMITTEE SHALL MAKE RECOMMENDATIONS TO THE DEPARTMENT REGARDING WHETHER
6 THE CARE PROVIDED WAS:

- 7 1. CONSISTENT WITH COMMUNITY STANDARDS.
- 8 2. CONSISTENT WITH COMMUNITY STANDARDS BUT OUTSIDE THE SCOPE OF
9 PRACTICE ~~[AS DEFINED BY THE DEPARTMENT IN RULE]~~.
- 10 3. OUTSIDE OF COMMUNITY STANDARDS AND REQUIRING CORRECTIVE ACTION.

11 ~~[K.]~~ [H.] FOR CARE DETERMINED TO BE OUTSIDE OF COMMUNITY STANDARDS
12 [OR THE SCOPE OF PRACTICE] UNDER SUBSECTION ~~[J-]~~ [G] OF THIS SECTION, THE
13 ADVISORY COMMITTEE MAY RECOMMEND ONE OR MORE OF THE FOLLOWING TO THE
14 DEPARTMENT:

- 15 1. ADDITIONAL CONTINUING EDUCATION REQUIREMENTS.
- 16 2. MENTORING OR SUPERVISED PRACTICE.
- 17 3. ADMINISTRATIVE CIVIL PENALTIES.
- 18 4. RESTRICTION OR SUSPENSION OF A LICENSE.
- 19 5. REVOCATION OF A LICENSE.

20 ~~[L.]~~ [I.] THE ADVISORY COMMITTEE SHALL REVIEW MIDWIFE REPORTS
21 SUBMITTED PURSUANT TO SECTION 36-763 AND MAY RECOMMEND MODIFICATIONS TO
22 REPORTING REQUIREMENTS TO THE DEPARTMENT TO SUPPORT QUALITY IMPROVEMENT
23 AND PATIENT SAFETY.

24 ~~[M.]~~ [J.] FOR THE PURPOSES OF THIS SECTION, "SENTINEL EVENT":

25 1. MEANS AN UNEXPECTED, SERIOUS PATIENT SAFETY INCIDENT RESULTING
26 IN DEATH, PERMANENT HARM OR A SIGNIFICANT RISK OF HARM.

27 2. INCLUDES BOTH:

28 (a) MATERNAL SENTINEL EVENTS, INCLUDING:

29 (i) MATERNAL ~~[DEATH]~~ [DEATHS DURING THE PREGNANCY THROUGH THIRTY
30 DAYS POSTPARTUM].

31 (ii) MATERNAL INTENSIVE CARE UNIT ADMISSION [IN THE FIRST ONE
32 HUNDRED TWENTY HOURS POSTPARTUM].

33 (iii) UTERINE RUPTURE.

34 (iv) POSTPARTUM HEMORRHAGE REQUIRING A BLOOD TRANSFUSION.

35 (v) SHOULDER DYSTOCIA WITH BRACHIAL PLEXUS INJURY.

36 (b) NEONATAL SENTINEL EVENTS, INCLUDING:

37 ~~[(i)]~~ [(i)] NEONATAL DEATHS IN THE FIRST THIRTY DAYS OF LIFE.]

38 ~~[(ii)]~~ [(ii)] NEONATAL INTENSIVE CARE UNIT ADMISSION WITHIN
39 [SEVENTY-TWO] [ONE HUNDRED TWENTY] HOURS AFTER BIRTH, NOT INCLUDING
40 OBSERVATION-ONLY ADMISSIONS OR ADMISSIONS FOR CONGENITAL ANOMALIES.

41 ~~[(iii)]~~ [(iii)] A BIRTH WEIGHT OF LESS THAN TWO THOUSAND FIVE
42 HUNDRED GRAMS.

43 ~~[(iv)]~~ [(iv)] AN APGAR SCORE OF LESS THAN SEVEN AT FIVE MINUTES.

44 ~~[(v)]~~ [(v)] RESPIRATORY DISTRESS REQUIRING PROLONGED VENTILATION.

1 36-763. Disclosure: reporting requirements: definitions

2 A. AT THE INITIATION OF CARE, EACH LICENSED MIDWIFE SHALL DISCLOSE
3 TO EACH PATIENT WHETHER THE LICENSED MIDWIFE MAINTAINS PROFESSIONAL
4 LIABILITY INSURANCE. THIS DISCLOSURE SHALL BE INCLUDED IN THE INFORMED
5 CONSENT DOCUMENT AND ACKNOWLEDGED BY THE PATIENT'S SIGNATURE.

6 B. ON OR BEFORE JANUARY 31 EACH YEAR, EACH LICENSED MIDWIFE SHALL
7 FILE A REPORT WITH THE DEPARTMENT, IN A FORMAT PRESCRIBED BY THE
8 DEPARTMENT, THE FOLLOWING INFORMATION, NOT INCLUDING ANY INDIVIDUALLY
9 IDENTIFIABLE HEALTH INFORMATION OF A PATIENT, FOR THE PRECEDING CALENDAR
10 YEAR:

11 1. THE NUMBER OF WOMEN FOR WHOM THE LICENSED MIDWIFE PROVIDED CARE.

12 2. THE NUMBER OF DELIVERIES THE LICENSED MIDWIFE ATTENDED AS
13 PRIMARY MIDWIFE.

14 3. THE NUMBER, REASON FOR AND OUTCOME OF EACH TRANSFER OR TRANSPORT
15 OF A PATIENT IN THE ANTEPARTUM, INTRAPARTUM OR IMMEDIATE POSTPARTUM
16 PERIODS.

17 4. THE NUMBER OF PERINATAL DEATHS, INCLUDING ~~[THE CAUSE OF DEATH~~
18 ~~AND A DESCRIPTION OF THE CIRCUMSTANCE]~~ [BOTH]:

19 (a) MATERNAL DEATHS OCCURRING DURING PREGNANCY AND THROUGH THE
20 FIRST MONTH POSTPARTUM.

21 (b) NEONATAL DEATHS OCCURRING THROUGH THE FIRST MONTH OF LIFE].

22 [5. FOR EACH DEATH REPORTED PURSUANT TO PARAGRAPH 4 OF THIS
23 SUBSECTION, THE CAUSE OF DEATH, IF KNOWN, AND A DESCRIPTION OF THE
24 CIRCUMSTANCES SURROUNDING THE DEATH.]

25 ~~[5.]~~ [6.] THE NUMBER AND OUTCOME OF BREECH BIRTHS AND VAGINAL
26 BIRTHS AFTER CESAREAN.

27 ~~[6.]~~ [7.] THE NUMBER OF BIRTHS THAT OCCURRED MORE THAN TWENTY-FIVE
28 MILES FROM A HOSPITAL THAT PROVIDES OBSTETRICS SERVICES.

29 ~~[7.]~~ [8.] THE NUMBER OF FETAL DEATHS AFTER TWENTY WEEKS'
30 GESTATION.

31 ~~[8.]~~ [9.] THE LICENSED MIDWIFE'S FULL NAME, LICENSE NUMBER AND
32 COUNTY.

33 C. IN ADDITION TO THE REPORTING REQUIRED IN SUBSECTION B OF THIS
34 SECTION, A LICENSED MIDWIFE SHALL REPORT TO THE DEPARTMENT:

35 1. A PERINATAL MORTALITY WITHIN SEVENTY-TWO HOURS AFTER THE DEATH.

36 2. A SENTINEL EVENT WITHIN FOURTEEN DAYS AFTER THE EVENT.

37 D. THE INFORMATION REPORTED TO THE DEPARTMENT PURSUANT TO
38 SUBSECTIONS B AND C OF THIS SECTION MAY BE INSPECTED, COPIED, OBTAINED OR
39 PROVIDED TO THE ADVISORY COMMITTEE FOR RESEARCH AND EVIDENCE.

40 E. THE REPORT REQUIRED BY SUBSECTION B OF THIS SECTION REPLACES THE
41 REPORT REQUIRED BY EACH LICENSED MIDWIFE FOLLOWING THE TERMINATION OF CARE
42 FOR EACH PATIENT.

1 [F. ON OR BEFORE SEPTEMBER 1 OF EACH YEAR, THE DATA THAT IS
2 COLLECTED IN THE REPORTS REQUIRED BY SUBSECTION B OF THIS SECTION SHALL BE
3 COMPILED AND SHARED WITH THE SPEAKER OF THE HOUSE OF REPRESENTATIVES, THE
4 PRESIDENT OF THE SENATE AND THE GOVERNOR AND POSTED ON THE DEPARTMENT'S
5 PUBLIC WEBSITE.]
6 ~~[F.]~~ [G.] FOR THE PURPOSES OF THIS SECTION:
7 1. "INDIVIDUALLY IDENTIFIABLE HEALTH INFORMATION" HAS THE SAME
8 MEANING PRESCRIBED IN SECTION 36-3801.
9 2. "SENTINEL EVENT" HAS THE SAME MEANING PRESCRIBED IN SECTION
10 36-762.
11 Sec. 4. Short title
12 This act may be cited as the "Jordan and Mack Terry Act".

13 Enroll and engross to conform
14 Amend title to conform

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